OCDYC-02 | OCDYC | Child aged 6–7 and a suitable support person

Ages 6–7: understanding a hard moment at home or elsewhere

1 | Purpose, fit and boundaries

Purpose of this unit

Gather a limited example of functional difficulty alongside the child's experience and supporter observation for specialist developmental and family assessment.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Repetition, bedtime rituals or routine preferences do not diagnose OCD. Do not repeatedly test the child or infer intent from thoughts. New changes, medical needs or concern about harm require separate assessment.

Reusing existing material

OCDYC is separate from ages 8 and over. Shortening an adult form does not establish suitability. Earlier OCDYC learning may help, but unit 2 does not prescribe a session number.

A fictional example for discussion

Fictional example: Tamar says leaving for kindergarten takes a long time. Her father separately describes events before the delay and help provided. The therapist examines context and impact before linking it to OCD.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

The therapist or support person reads one question. The child may use a few words, point or skip. Adults may write the child's words but not answer for them. Professional terms and descriptions of every thought are not required.

1 | Question for discussion

When was it hard: at home, kindergarten or elsewhere?

2 | Question for discussion

What did I want to do then?

3 | Question for discussion

What did I do when it got hard?

4 | Question for discussion

Who was with me and what did they do?

5 | Question for discussion

What would I like to be easier in my day?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

Accept not knowing why the child acted. Supporters describe observations without seeking proof of diagnosis. Differences between reports are for exploration.

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Child, support person and therapist — separate roles

3 | Ages 6–7 only; developmental and family adaptation required

The voice of the child aged 6–7

Use a simple before-and-after sequence. Do not ask abstract questions about a ritual's function.

The parent or support person's role

Give one example including context and your response. Avoid continuous monitoring or labeling behavior a ritual before assessment.

The therapist's role and adapting the setting

Use professional assessment to distinguish developmental needs, other difficulties and an OCD cycle. This form is neither a screening tool nor a validated questionnaire.

Limits of the material

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

> Do not apply this pathway to other age groups without separate review.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

The P sheet is shared: two questions for the child's voice and three for supporter observations. Use only as agreed with the therapist, without child-alone exposure homework, abrupt response changes or punishment. Daily records and thought counts are unnecessary.

At the next session

Check whether functioning is clearer and what needs further assessment. Reduce recording if it increases interrogation or reassurance requests.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

www.nice.org.uk/guidance/cg31/chapter/Recommendations
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